SAINT MICHAEL COLLEGE OF CARAGA

RECORDING RATHER THAN RESCUING: UNDERSTANDING THE DETERMINANTS OF BYSTANDERS APATHY DURING EMERGENCY SITUATION

A Thesis Proposal Presented To
The Faculty of
College of Criminal Justice Education

In Partial Requirements for the Degree
BACHELOR OF SCIENCE IN CRIMINOLOGY

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OCTOBER 2025

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CHAPTER 1

THE PROBLEM AND ITS SCOPE

1.1 Background of the study

A significant problem in society is that some people record emergencies instead of helping those who are hurt. This is especially true now that so many people use smartphones. This practice is important for identifying the psychological, social, and environmental factors that affect individuals who are not directly involved in an emergency. Today, the reasons why some people choose to record instead of rescue are important because they affect how emergencies unfold, public safety, and social responsibility. Documenting events can provide valuable evidence and raise awareness, but inaction can directly lower survival rates for victims and make people less likely to act during a crisis. This is a response to changes in technology and culture that affect how people act and what the rules are.

A significant amount of research has been conducted on the actions bystanders can take in emergencies worldwide. Studied more than 16 million EMS calls in the US and found that bystander intervention happened in 11% of cases. The likelihood of intervention depended on the patient's characteristics, the type of injury, and the environment. It is important to note that more help was offered during life-threatening emergencies compared to sensitive emergencies like sexual assault (Faul et al., 2016). The presence of people significantly reduced the number of rescues that could be done, which is similar to the bystander effect. These studies shed light on some important behavioral trends, but they do not examine the recording aspect, which is becoming increasingly important with the rise of cell phones. Others have suggested that fear of the law, social judgment, and the sharing of responsibility make people less likely to help;

however, there is limited real-world evidence on the recording of behavior (Kiyohara & Mizu, 2025).

Locally, studies have also demonstrated that cultural and social nuances influence bystander behavior. Recent mixed-methods studies in Southeast Asia found that people do not help in emergencies because they are afraid of getting in trouble. After all, it is socially acceptable not to, or because they do not know how to do first aid. Local research, on the other hand, typically only examines intervention rates or attitudes, rather than the distinction between recording and rescuing. Further research is needed on how new technologies and digital culture influence bystanders' choices in real-world situations. Contextual work that combines psychological, social, and technological factors is so important. One important area of research revisits the bystander effect. The bystander effect occurs when people are less likely to help when others are present (Hortensius & de Gelder, 2018).

Due to these gaps and the need to quickly learn more about why bystanders record but do not intervene, this research was conducted. Solving this problem will help us understand the causes of this widespread trend, taking into account factors such as fear, social influence, legal issues, and the effects of new media. The study aims to fill the gaps left by earlier studies by examining issues from both local and global perspectives, moving beyond traditional models that focus solely on rescue to gain a deeper understanding of recording behavior. To balance the benefits of recording with proactive measures that save lives, this approach can help guide interventions aimed at making people more responsive and educating them.

Lastly, this highlights the importance of understanding what causes people to disengage during crises. The study aims to contribute to the academic literature and support emergency

management professionals by examining the psychological, social, and environmental factors that influence individual reactions. The results will help inform specific interventions and campaigns aimed at encouraging people to help others, which should reduce the bystander effect. Lastly, this work benefits society as a whole by making it more responsible and responsive, and it also benefits individuals by providing them with more Information that they can use immediately in times of need.

1.2 Theoretical Framework

The Bystander Effect theory, developed by John M. Darley and Bibb Latané in 1968, serves as the foundation for this study. The theory suggests that when others are present, individuals are less likely to help a victim. The witnesses are now responsible for what happened. Because of this sharing of responsibility, each person will feel less obligated to act because they think someone else will step in. The killing of Kitty Genovese is used as an example of this effect in historical studies. Several people who were nearby are said to have done nothing to help or call the police. They found three psychological processes that might stop bystanders from helping people who are in trouble: evaluation apprehension (fear of being judged in public), plastic Ignorance (the tendency to rely on the obvious reactions of others when describing an unclear situation), and diffusion of responsibility.

This theory can help us understand why modern bystanders choose to record emergencies instead of helping. The behavior of bystanders (recording or rescuing) is directly influenced by the independent variables of social presence, perceived responsibility, and fear. Having other people around makes you feel less responsible for helping. It increases the likelihood of doing nothing or taking indirect action, such as recording the incident, rather than taking direct

action. Bystanders' concerns about being punished by the law or socially judged for actively rescuing someone are supported by psychological factors, such as evaluation apprehension.

Pluralistic Ignorance can also explain why bystanders observe what others do to determine if they need to intervene and help, which could mean doing nothing if group norms suggest that recording is preferable to saving. So, this framework clearly explains the social and psychological processes at play. It also provides a starting point for exploring how new technologies, such as smartphones, can increase the likelihood of people recording events rather than acting on them, which ties together all of the study's main variables.

In short, the Bystander Effect theory fully explains the phenomenon under study by explaining how people tend to over-rescue in emergencies because they feel less responsible and are constrained by social and psychological norms. By examining the relationships between social presence, perceived risk, fear, and the choice between action and inaction, we can gain a deeper understanding of what motivates bystanders to take action. We are doing this research on the social and technological factors that affect how people respond to emergencies.

1.3 Review of Related Literature

A review of both foreign and local literature reveals that several factors influence bystander behavior in emergencies. The researchers will examine the following aspects to determine why some people prefer recording over rescuing and how societal and technological changes influence this behavior. With this knowledge, efforts can be made to encourage people to help others in times of need, rather than focusing on taking pictures for social media.

Age

There are several ways in which age affects bystander apathy. An experiment conducted by Laila Song in 2024 found that older teenagers (tenth graders) were significantly more likely to assist in emergencies than younger children (first graders), who were less likely to offer help. This suggests that people's responses to emergencies and societal evolution play a role. However, more general reviews have yielded mixed findings regarding the effect of age, with some studies showing no effect at all and others suggesting that older people may be more likely to intervene, depending on the situation (Mainwaring et al., 2022).

When children are young, they exhibit basic prosocial tendencies, but the social situation still influences their behavior. A study by Plötner et al. (2015) demonstrated that even five-year-old children exhibited the bystander effect, indicating that they were less likely to help when their friends were present. This finding shows that the spreading of responsibility can happen even at a very early stage of development. Showing that age alone does not guarantee helpful behavior.

The teenage years and early adulthood are periods when people are more susceptible to social judgment and peer pressure, which can make them less likely to lend a helping hand to others. Yüksel et al. (2022) found that teenagers are more likely to ask their peers for help than to ask adults directly. Illustrating the significance of social context and concerns about being judged at this stage of life. Explaining why younger people today prefer indirect or passive responses, such as recording incidents on their phones instead of helping, is that they are afraid of being embarrassed or facing dire social consequences.

Ma et al. (2023) suggest that as people age and become adults, they tend to help others more because they have developed better moral reasoning, empathy, and a greater sense of their

own competence. People over 65 often feel more obligated to help others, but they may not be able to assist due to physical limitations or concerns about taking on direct risk Faul et al. (2016). Also, the rise of social media has made "recording" common among younger bystanders, who see it as a way to take part without putting themselves in danger (Hortensius & de Gelder, 2018).

Gender

Gender has a significant impact on how bystanders respond when they need to intervene. In situations involving violence or victimization, women are more likely to show empathy and step in, according to research. Deacon's (2020) study found that male bystanders had less empathy and more negative attitudes, like transphobia, which made them less likely to help than female bystanders. Perceived barriers to intervention are also affected by gender and empathy. In sensitive situations, women are more likely than men to want to help (Yule et al., later review). How people are socialized and how they feel about things affect bystander apathy.

To understand how bystanders behave in emergencies, research has found that gender is a significant factor. Studies show that differences in how people help others are often caused by social roles, perceptions of the danger involved, and their attitudes toward their own gender. Fischer et al. (2011) conducted a meta-analysis. They found that when situations are perceived as dangerous, men are more likely to step in and provide direct help. At the same time, women are more likely to seek help indirectly or avoid physical involvement. This difference comes from traditional ideas about gender roles and worries about personal safety.

Levine et al. (2002) expanded on this idea by demonstrating that female bystanders were more likely to help other women when they felt they had something in common with them—suggesting that a person's gender identity can make them more empathetic and helpful. As

smartphones and social media become increasingly popular, the way people exhibit bystander apathy has changed.

Researchers García and Rendón (2020) looked at how men and women record during emergencies and found that men are more likely to record violent or upsetting events for social recognition, while women are more likely to record for documentation or personal safety. Those results show that gender affects not only the choice to help, but also the reasons for recording instead of saving.

Frequency of Social Media Usage

People's actions in real and virtual emergencies are affected by how they use social media. Pittaro (2019) suggested that individuals who use social media extensively may become desensitized to emergencies, potentially leading them to act less proactively, such as recording incidents rather than providing assistance. This trend is thought to be caused by the sharing of responsibility online and the desire for fame or attention from peers. However, some studies (recent reviews from 2023) show that personal safety concerns and social media presence do not have a big effect on bystander apathy.

More research is being conducted to determine the impact of online social environments on intervention. The rise of smartphones and social media over the past few years has altered how people respond to emergencies. Many people who are not involved in the event now record it and share it on social networking sites like Facebook, TikTok, or X (formerly Twitter). Thomas et al. (2020) referred to this phenomenon as the "recording reflex," which occurs when people automatically take pictures of crises with their phones, prioritizing digital records over direct assistance.

Garcia and Rimé (2019) said that recording can be a way to deal with mental health problems by separating people emotionally from upsetting scenes. They feel like they are part of the process by observing and talking, but they do not have to take the immediate risk or feel morally responsible to help. Aligning with the concept of performative empathy, which posits that people exhibit concern or sympathy online to garner attention or Approval rather than genuinely helping (Rosen et al., 2013).

The way people react emotionally and behaviorally in emergencies is directly related to how frequently they use social media. Researchers Vossen and Valkenburg (2016) found that exposure to a large amount of emotionally charged content online can lead people to become less sensitive to or wearied of empathy for real-life suffering. People may become passive observers of tragic events over time, viewing them as normal, routine, or even interesting, rather than as important.

Kross et al. (2013) noted that spending a significant amount of time on social networks, such as Facebook, is associated with lower empathy and poorer emotional health. Additionally, Nabi et al. (2018) introduced the concept of "compassion fatigue" in the digital age, which occurs when repeatedly viewing upsetting content diminishes one's motivation to be kind to others in real life. This emotional numbness makes people more likely to record or post content instead of taking action to rescue those in need.

Regularly using social media not only makes people less empathetic, but it also encourages moral disengagement —a process by which people find reasons to justify doing bad things. Thomas et al. (2020) found that individuals who frequently use social media are more likely

to claim that they do not need to take action, as they believe that sharing videos helps spread awareness. However, these kinds of excuses are often just ways to avoid taking real responsibility.

Chaudhry and Gruzd (2019) also noted that social media encourages people to seek attention, and that users obtain Approval through likes and shares, sometimes even by capitalizing on sensitive situations. The desire to be seen online can take precedence over moral urges to help people in danger, turning emergencies into opportunities to create content.

According to Simon et al. (2015), social media sites have become increasingly important during emergencies because they enable people to share Information, pictures, and videos in real-time. However, being connected all the time can also make it normal to watch. People may think that posting about or documenting an event online is their social duty, which makes them less likely to offer direct help. In this way, social media exacerbates the "bystander effect" by encouraging people to focus on watching rather than taking action.

A related study by Bastiaens et al. (2018) looked at bystander intervention in abusive online situations. They discovered that people who use social media a lot were more likely to do nothing. Researchers have found that people become less sensitive to upsetting content on social networks, such as Facebook and Twitter, as they use them more frequently. They believe that stepping in is either unnecessary or could be socially risky. Aligning with the broader trend of "recording rather than rescuing," which suggests that individuals who spend a significant amount of time online are more likely to observe rather than take action.

Additionally, the Brookings Institution reported in 2021 that, despite more than 90% of teens and young adults encountering harmful or upsetting content online, fewer than 40% take action or report it. Using social media a lot makes you more aware of emergencies, but it does not

always make you more empathetic or involved. Instead, constant engagement can cause people to mentally separate from real-world crises, viewing them as things to be consumed rather than urgent moral events that require action.

Bennett et al. (2021) found that individuals who regularly record instead of helping tend to justify their actions by claiming they are doing something to raise awareness or do what is right. However, these kinds of justifications often exhibit moral disengagement, which occurs when one puts off taking responsibility for their actions and blames social or digital systems. Over time, this kind of behavior becomes normalized, leading to passivity and weakening community bonds.

Prior First Aid Knowledge

Knowing how to give first aid usually makes people more willing to help. Studies show that trained individuals are more confident in themselves and their ability to help in emergencies, which reduces their fears of hurting someone or being unable to perform their job effectively. For instance, Duut et al. (2022) found that bystanders were more likely to help if they had more knowledge about first aid and held a positive attitude towards it. Training programs make people feel more responsible and equip them with additional skills. This makes it easier for others to assist, which in turn leads to more helping behavior.

Riccò et al. (2020) stated that learning first aid and CPR is one of the most effective ways to encourage people to help others. Their research showed that people who had already received training were significantly more willing to help with CPR, being a bystander, and other first aid tasks. People who did not have this kind of training, on the other hand, were more likely to stay passive observers—demonstrating how competence and knowledge can help individuals overcome the psychological barriers that prevent them from taking action. This evidence suggests

that training raises self-efficacy, or the belief that one can effectively respond. Crucial for encouraging people to be proactive in emergencies.

To support this, Van de Velde et al. (2009) examined the effectiveness of non-resuscitative first aid training programs. They found that individuals who had received training were more proficient and more confident in performing emergency tasks. Being prepared in this way reduces feelings of helplessness and fear, which are common triggers for bystander apathy. The study concluded that learning first aid improves both technical skills and emotional readiness, enabling people to transition from being passive observers to active responders.

Mao et al. (2021) also conducted a cross-sectional study that showed people who knew about first aid, had practiced it, and felt confident about it were much more likely to be willing to help bystanders by taking actions such as checking if the person is still conscious or calling for help. Suggest that individuals with formal training in first aid or CPR are more willing and confident to help others in an emergency than those without such training. Their research showed that training improves both knowledge and situational awareness, making people less hesitant and less afraid of making mistakes. Being prepared enables bystanders to act quickly and make informed decisions when faced with extreme stress, which is crucial in life-threatening situations where the speed of response can significantly impact survival chances.

Similarly, Mobarak, Afifi, and Qulali (2015) emphasized that students who have received first aid training are more likely to want to help others and believe in their own abilities. People who know what to do in an emergency are less likely to panic and more likely to take action on their own, rather than waiting for someone else to help. Both studies also show that learning first

aid not only improves technical skills but also enhances mental preparedness and moral responsibility, making people feel that it is their civic duty to help those in need.

Abdelrahman et al. (2024) also stated that receiving proper first aid training not only enhances technical skills but also fosters moral and social responsibility, as well as a sense of duty to assist those in need. Their study of teachers in Riyadh highlighted the importance of ongoing first aid programs in enhancing both readiness and safety within the community. Collectively, these studies demonstrate that knowing first aid helps bridge the gap between wanting to help and actually doing so. Making bystanders more likely to help in an emergency reduces their reluctance to do so.

Exposure to Emergencies

Bystander apathy can also be changed through direct or indirect exposure to emergencies, often by altering how people feel and react emotionally. Hortensius et al. (2016) found that personality traits, such as personal distress, influence automatic responses to emergencies and the likelihood of intervening when others are present. People who are really upset might freeze up or pull away in a group setting, which stops them from helping others. Knowing how to handle emergencies can influence how likely someone is to step in, taking into account both personality and situational factors.

Bystanders' responses to emergencies are significantly influenced by their level of exposure to such events through direct experience, witnessing incidents, or participating in training and drills. Individuals who have experienced a similar situation before are more likely to perceive it as an emergency, act quickly, and feel a sense of responsibility. People who have not experienced much or anything may freeze, hesitate, or act passively, such as recording instead of

intervening. Chew et al. (2019) discovered that people who had witnessed a family member's cardiac arrest (a type of emergency exposure) were much more likely to perform bystander CPR than people who had not witnessed such an event. The authors said that previous exposure makes people more willing to "pay it forward" and take action.

Additionally, Faul, Aikman, and Sasser's (2016) large-scale real-world data revealed that bystander intervention rates remain low (11%) and vary significantly depending on the type of emergency. Suggesting that being present is not enough to define action. Hall's (2016) qualitative grounded theory study of lay decision-making also supports the link between exposure and action: participants who had experienced emergencies or simulation drills prior to the study felt more confident in their ability to handle real emergencies and were less emotionally paralyzed when they occurred. In modern situations, where smartphone cameras may lead people to watch instead of help, prior exposure becomes increasingly important. If a bystander has experience, they see emergencies as opportunities to act. If they do not, they are more likely to record what is happening. Being exposed to emergencies is a key factor that affects bystander apathy and the change from "recording" to "rescuing."

Diffusion of Responsibility

Diffusion of responsibility is a psychological phenomenon in which people feel less obligated to act in emergencies when others are present. The reason for this is that all witnesses feel responsible, so they believe someone else will step in and stop the crime. As a result, the likelihood of receiving direct help decreases as the number of observers increases.

Kawamichi et al. (2016) suggest that when people act in groups, their sense of agency and accountability tends to weaken. Making them less emotional slows down their decision-

making in important situations. Hortensius and de Gelder (2018) also said that when people think others are nearby, their brain's action and empathy systems slow down, which makes it take longer for them to act. Because of this, people in crowds are more likely to record what is happening than to provide direct assistance. The responsibility is shared instead of being personal.

Diffusion of responsibility can occur in both digital and real-world settings. Fischer et al. (2018) found that people on social media often hesitate to report or help when they see online emergencies because they think that someone else will do it. This pattern aligns with the trend of "recording rather than rescuing," which means that people nearby choose to watch rather than help because they believe others will.

Additionally, more research supports this, showing that people are less likely to step in when they think that responsibility is shared among a group. Britannica (2016).

New research in neuroimaging and behavioral science also suggests that the spreading of responsibility is linked to reduced activity in brain regions responsible for assisting others and making moral decisions (Beyer, 2016).

Evaluation Apprehension

People with evaluation apprehension worry about how they will behave in social situations because they are afraid of being judged or negatively evaluated by others. In an emergency, this psychological response can cause people to hesitate or fail to act because they are afraid of how others will react. As Meyers (2016) points out, this fear of being judged by others is a big reason why many people do not offer direct help, even when they know someone needs it.

Fischer et al. (2018) suggest that bystanders often consider the social consequences of intervening, such as embarrassment, making a mistake, or being criticized, before taking action. Being self-aware makes people more hesitant, especially in public or crowded places where they think someone is watching them. Garcia and Weaver (2019) also noted that people are more likely to act when they believe others will approve, but they are less likely to act when they are unsure of how others will react.

The widespread use of smartphones and cameras in today's digital world makes evaluation anxiety even worse. Many bystanders do not act directly because they are afraid of being caught on camera and judged online. Instead, they record the event. Faklaris et al. (2020) found that people often choose to film emergencies instead of being there in person because they are afraid of being judged by others or being wrongly accused of breaking the law. As the theme of "Recording Rather than Rescuing" suggests, this aligns with the idea that bystanders should capture images instead of intervening directly.

People who worry about being judged negatively if they step in during an emergency are said to have evaluation apprehension. This factor has been shown to significantly reduce people's willingness to help, especially in social situations where individuals believe others will closely observe them. For instance, studies show that people who witness a crime often do not intervene because they are afraid of making a mistake or being judged harshly (Fischer et al., 2011).

However, Safoura Jahedizadeh et al. (2023) said that evaluation apprehension is the stress that comes from worrying that other people will think badly of your knowledge or skills. They reviewed 30 studies from a diverse range of disciplines and found that this concept has multiple

levels of complexity. They also found that the fear of being judged negatively can make it difficult to perform well and participate in many situations.

New studies confirm that the fear of being judged is a primary reason why people do not take action. According to Hortensius and de Gelder (2018), bystander apathy is caused by more than just shifting blame. It is also caused by problems with self-regulation and fear of being judged. Their research indicates that individuals who fear being judged by others tend to suppress their emotions, even when they recognize the need for help.

Similarly, Luo et al. (2023) investigated how the social distance between bystanders affects the degree of embarrassment and their willingness to act. Their research showed that people are more afraid of being judged negatively when they are with strangers, which makes them less likely to do anything. On the other hand, people who were around familiar observers felt less afraid of being judged negatively. Neuroscientific evidence supports the idea that evaluation anxiety affects how bystanders act. Researchers have found that this fear activates areas of the brain involved in self-consciousness and social judgment. These include the medial prefrontal cortex and the anterior cingulate cortex, which regulate emotions and prevent people from intervening on their own (Tang et al., 202).

These brain responses show that helping others is not just a moral choice; it is also a mental process that is affected by how others see you. Hortensius et al. (2016) also found that individuals who are more distressed and anxious about social situations are less likely to intervene when others are present. This is because their increased emotional arousal makes them more focused on themselves and worries about what other people will think of them.

In line with these results, Liebst et al. (2019) demonstrated that the social dynamics of the group significantly influence bystander intervention. People are less likely to help when they think they are being judged or when there are not many strong social bonds between bystanders.

Fear of being evaluated has an impact on both digital settings and real-life emergencies. Mainwaring, Scott, and Gabbert (2024) discovered that people who are afraid of social rejection and damage to their reputation are less likely to report or help with cases of image-based sexual abuse. Sjögren et al. (2024) also said that moral disengagement and low bystander self-efficacy make people less likely to want to help others. Demonstrating that cognitive justifications for not acting and internalized fear of being judged operate in tandem. According to these findings, evaluation apprehension is a complex idea that involves mental, emotional, and social factors that work together to stop people from helping others in both real and virtual settings. To address this issue, we require programs that enhance social confidence, empower bystanders to believe in their own abilities, and normalize people stepping in as a socially acceptable response, rather than a threat to their reputation.

Pluralistic Ignorance

Pluralistic Ignorance is a psychological phenomenon in which people believe that others' lack of response indicates that they do not need to help or take action. This way of thinking makes people less likely to help in emergencies, which makes the group less active. Pluralistic Ignorance occurs in the context of the bystander effect, where people misinterpret others' calm or quiet behavior as a sign that no one needs help, thereby delaying or preventing help from being given.

Indeed, Rendsvig (2014) states that pluralistic Ignorance in groups leads to the assumption that everyone else knows more about what is happening. This mistake leads to much hesitation,

especially in emergencies where the situation is unclear. Meyers (2016) also noted that when people do not see others caring, they tend to act in a way that matches how the group thinks they should, even if their own instincts tell them to intervene.

New studies continue to investigate this link in modern times, particularly now that an increasing number of people use smartphones. Faklaris et al. (2020) found that in public places, people often choose to record or livestream emergencies rather than provide direct assistance. The influence of technology and the desire to conform to others. This behavior is a form of pluralistic Ignorance, where people think that filming is the right thing to do because other people are doing it. It has become normal to record instead of rescue, which reflects a shift in society where watching is replacing participation.

Ombayan et al. (2025) also found that students' frequent use of cell phones alters their interactions with others and their reaction times in public. Their results suggest that social norms surrounding technology can influence bystander behavior, increasing the likelihood that they will do nothing when an emergency occurs. All of these studies demonstrate that pluralistic Ignorance remains a significant psychological factor that contributes to bystander apathy, particularly in today's digital world of connectivity.

Recent reviews suggest that social norms and the setting in which people observe others reacting make this process stronger, creating a cycle of Ignorance that makes it harder to help. Neuropsychological research suggests that this shared false belief engages parts of the brain that facilitate social reasoning and conformity, making people even less likely to act (Hortensius, 2018). Understanding pluralistic Ignorance and addressing education and clear communication can significantly reduce the effects in many situations.

New ideas about pluralistic Ignorance suggest that it is a process that evolves in response to societal influences and emotional factors. Hortensius and de Gelder (2018) stated that bystander apathy is caused by a combination of various social, emotional, and neural processes that collectively shape how people respond to emergencies. They said that seeing someone else stay calm or passive in a potentially dangerous situation triggers a neural and emotional conflict in the observers: social cues indicating that the situation may not require action override the observers' natural desire to help and show empathy. This process illustrates how emotional control is socially mediated: people assess the seriousness of something based on how others react, and if no one reacts quickly, they conceal their own worries to conform to what they believe the group expects. So, apathy in bystander situations is not just because people do not care; it is also caused by social-emotional synchronization, which means that when everyone is calm, it is mistakenly perceived as proof that no intervention is needed.

Similarly, Miller (2023) noted that public cues and social signaling that alter how people perceive group norms perpetuate pluralistic Ignorance. People who observe many individuals not taking action often assume that others believe the situation is not urgent or does not require help, even if they personally believe help is needed. This false belief leads people to act in a manner that aligns with what they believe the majority thinks, perpetuating the cycle of silence and inaction. Miller also said that pluralistic Ignorance grows when people are afraid of being socially shamed for overreacting or misjudging a situation, which makes them hide their real concern to keep the peace. In modern emergencies, such as those caught on camera and shared on social media, these dynamics are amplified: the fact that passively recording people and seeing them without reacting can make it appear that getting involved is socially unacceptable. These studies

demonstrate that bystander apathy is not merely a lack of empathy; it is also a learned social inhibition resulting from emotional conformity and misconceptions about group norms.

One sign of pluralistic Ignorance in modern society is the growing trend for bystanders to record emergencies instead of helping directly. Andersson (2021) discussed how watching others film instead of helping creates a social script that normalizes documentation as the appropriate response, shaping how people perceive they should act in a crisis. Van Bommel et al. (2020) did experiments that showed people are less likely to want to help when they know others are watching, especially when actions could be seen by everyone or recorded for social media. People perceive others' lack of action as a sign that they do not need or should not be helped. Rizzo (2023) also noted that filming allows bystanders to demonstrate their involvement without taking the personal or social risks associated with physically intervening. It gives people a way to appear concerned without risking criticism or failure.

1.4 Schematic Diagram

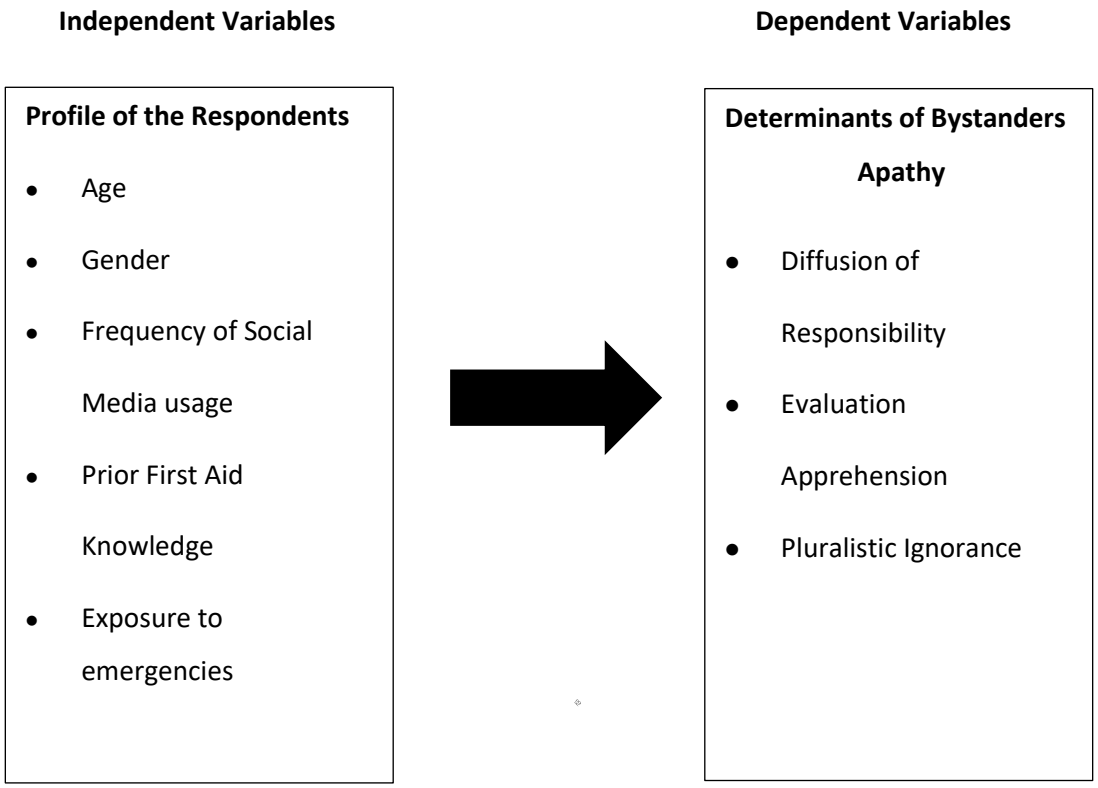


Figure 1. This Schematic Diagram shows the interplay of independent and dependent variables of the study.

1.5 Statement of the Problem/ Objectives of the Study

This study aims to understand the factors that contribute to bystanders' apathy during emergencies.

Specifically, the study seeks to answer the following question:

1. What is the demographic profile of the respondents in terms of:

1.1 age;

1.2 gender;

1.3 frequency of social media usage;

1.4 prior first aid knowledge; and

1.5 exposure to emergencies?

2. What factors contribute to bystanders' apathy during emergencies in terms of:

2.1 diffusion of responsibility;

2.2 evaluation apprehension; and

2.3 pluralistic ignorance?

3. Is there a significant relationship between the respondents' demographic profile and the factors that contribute to bystanders' apathy during emergencies?

1.6 Hypothesis

The following hypothesis guides this study:

Ho1. There is no significant relationship between the profile of the respondents and factors that contribute to bystanders' apathy during emergencies.

1.6 Scope and Limitations

This study examines the factors that contribute to bystanders' apathy during emergency situations. It is limited to selected respondents from Buenavista, Nasipit, and Carmen, who may have encountered an emergency situation, such as road accidents, medical emergencies, physical altercations, fires, or similar crises in which they were not the primary victim but were present at the scene and choose to record rather than help. Will be use purposive sampling to identify the respondents. The data will be collected through a survey questionnaire to gather insights into their perceptions and behaviors. The study will be conducted during the first and second semesters of the 2025–2026 school year.

1.8 Definition of Terms

The following terms and statements were operationally defined based on their use in this study.

Age - Refers to the number of years a respondent has lived since birth. In this study, participants are categorized by their chronological age to investigate the relationship between their age and bystander behavior during emergencies.

Apathy -Refers to a lack of motivation, concern, or emotional responsiveness toward a situation. It describes the indifference or reluctance of bystanders to provide help or intervention during emergencies.

Bystanders - Individuals who witness or are present during an emergency but are not directly involved in the event or the victim's condition. Bystanders may either take action, such as rescuing or calling for help, or remain passive by observing or recording the incident.

Bystander Effect - Refers to a social psychological phenomenon where individuals are less likely to help a victim when other people are present. It serves as the theoretical basis for analyzing the factors that influence bystanders' likelihood of intervening or remaining passive during emergencies.

Emergency Situation - Refers to an unexpected and urgent event that poses an immediate threat to life, health, property, or the environment. It pertains to incidents that require quick decision-making and immediate intervention from bystanders.

Evaluation Apprehension - Refers to the fear or anxiety individuals experience when they believe others will negatively judge their actions. It relates to the hesitation of bystanders to intervene due to concerns about making mistakes or facing social criticism.

First Aid Knowledge - Refers to the awareness, understanding, and ability to perform basic medical procedures before professional help arrives. It represents a factor that influences the bystander's confidence and readiness to assist with emergencies.

Gender - Refers to the biological classification of respondents as male or female. It is considered to examine possible differences in bystander responses and intervention tendencies between genders.

Recording refers to the act of capturing or documenting an emergency using electronic devices, such as smartphones or cameras.

Rescuing - Refers to the deliberate act of providing aid or assistance to individuals in distress during an emergency. This includes actions such as administering first aid, contacting emergency services, or offering physical help to victims.



CHAPTER 2

RESEARCH METHODOLOGY

This chapter presents the research design, research locale, research respondents, research instrument, ethical standards, data gathering procedures, and data analysis plan used in the study.

2.1 Research Design

This study employed a descriptive quantitative research design, focusing on the characteristics, behaviors, and perceptions of bystanders during emergencies using numerical data. The descriptive method involves systematically collecting, classifying, analyzing, and interpreting data on existing conditions, practices, attitudes, and relationships. It aims to provide an accurate representation of the current situation without manipulating any variables.

Data were gathered using a structured survey questionnaire distributed to respondents from Buenavista and Carmen. This method allows researchers to collect measurable data that can be statistically analyzed to identify patterns, relationships, and trends among the variables under study. Through this approach, the study aims to identify the factors influencing bystanders' apathy and responses during emergencies.

According to Creswell and Creswell (2018), quantitative research enables researchers to collect and analyze numerical data to explain phenomena, test relationships, and draw conclusions that can be generalized to a larger population. This design ensures objectivity, accuracy, and reliability in the interpretation of results.

The descriptive quantitative design was therefore deemed appropriate for this study, as it allows the researchers to present a factual and comprehensive account of bystanders' behavior during emergencies. The results will serve as the basis for interpreting trends and understanding the factors that contribute to bystander apathy.

2.2 Research Locale

This quantitative research investigates the factors that contribute to bystanders' apathy during emergencies in the municipalities of Buenavista, Nasipit, and Carmen, Agusan del Norte. It seeks to understand why some individuals choose to record incidents rather than provide help. The study will gather data through interviews and focus group discussions with residents, first responders, and local officials to identify the psychological, social, and cultural factors that affect bystander behavior.

2.3 Research Respondents

This study will be conducted in the municipalities of Buenavista, Nasipit, and Carmen, located in the province of Agusan del Norte. These areas were selected because of their growing population, increasing public activities, and frequent occurrence of community incidents where researchers can observe bystander behavior. The setting provides an ideal environment to understand the determinants of bystanders' apathy during emergencies. The respondent will be residents who have witnessed or encountered emergencies such as accidents, crimes, or natural disasters. Barangay officials and local responders, police officers, barangay tanods, or rescue volunteers who have observed public reactions during emergencies. Youth and students in the area who are active social media users and may have been involved in recording incidents.

2.4 Research Instruments

The primary tool used in this study is a self-developed questionnaire, created by the researchers to gather quantitative data on the determinants of bystanders' apathy during emergencies. The instrument will be designed based on the study's review of related literature and stated research objectives. It will be composed of two main parts:

Part I – Respondents Profile

This part will obtain the demographic and experiential Information of the respondents, including age, gender, frequency of social media usage, prior first aid knowledge, and exposure to emergencies. These factors will serve as the independent variables that may influence bystanders' apathy during emergencies.

Part II – Determinants of Bystanders' Apathy

This part will measure the dependent variables that represent the possible determinants of bystanders' apathy. It will consist of statements categorized under three components:

Diffusion of Responsibility – will assess the tendency of individuals to rely on others to take action during emergencies.

Evaluation Apprehension – will evaluate the fear of being judged or embarrassed when offering help.

Pluralistic Ignorance – will measure the tendency to interpret others' inaction as a sign that intervention is unnecessary.

Each statement will be rated using a four-point Likert scale, with the following scale interpretation: 4 – Strongly Agree, 3 – Agree, 2 – Disagree, and 1 – Strongly Disagree. The questionnaire will undergo expert review to ensure the content validity, accuracy, and clarity of the items. In addition, a pilot test will be conducted to assess the reliability and internal consistency of the instrument using Cronbach's Alpha before its final distribution to the respondents.

2.6 Research Ethical Standards

In this study, entitled *"Recording Rather than Rescuing: Understanding the Determinants of Bystanders' Apathy During Emergency Situations"*, the researchers strictly adhered to ethical standards to maintain integrity, honesty, and respect for all participants. The following ethical considerations were observed:

A. Protection of Intellectual Property Rights (IPR)

This research did not involve the creation of new technologies or products requiring patent protection. However, all reports, surveys, and self-developed questionnaires are considered original works and are protected by copyright. Intellectual property rights were respected, and all references, logos, and materials used were cited correctly and acknowledged.

B. Informed Consent

Since this research involved only human participants, each participant provided informed consent prior to participation. The consent form explained the study's purpose, objectives, possible risks, and benefits. Respondents were informed that participation was voluntary and that

they could withdraw at any time without consequences. For minors, consent was also obtained from their parents or legal guardians.

C. Data Privacy and Confidentiality

All collected data, including personal and sensitive Information, was treated as strictly confidential. Respondents' identities were kept anonymous, and all data were securely stored and accessible only to the researchers. Researchers used Information gathered solely for academic purposes.

D. Voluntary Participation and Freedom to Withdraw

Participation in this study was completely voluntary. Respondents had the right to refuse or withdraw from the study at any time without penalty or consequence. Researchers applied no form of coercion or pressure. As the study involved only human participants, animal-related ethical concerns were not applicable.

E. Minimization of Harm and Risk Management

Since the study's topic could evoke emotional or sensitive reflections about real-life emergencies, the researchers ensured that no question would cause mental, emotional, or social distress. Any respondent who felt uncomfortable could skip questions or withdraw from participation at any time. The researchers approached all participants with empathy and respect to protect their well-being throughout the process.

F. Beneficence and Contribution to Knowledge

This study aimed to provide meaningful insights into the psychological and social factors influencing bystanders' apathy during emergencies. The findings aim to raise public awareness, promote responsible actions during crises, and support safety programs and educational initiatives. Results were shared with relevant institutions to contribute to knowledge and improve community response during emergencies.

G. Justice and Fair Participant Selection

Respondents were selected fairly in accordance with the study's inclusion criteria. No participant was excluded based on gender, social status, or any other discriminatory factor. Equal opportunity was given to all qualified respondents.

H. Data Integrity and Accuracy

All data were collected and analyzed honestly and objectively. Researchers corrected any inconsistencies identified during the process to maintain the accuracy and reliability of the findings. The researchers ensured that all interpretations reflected genuine results without bias or distortion.

I. Transparency and Honesty in Reporting

Researchers presented all findings in this study truthfully and accurately. Researcher: Clearly describe the methods, data, and results to ensure the research can be understood and verified by others. Artificial intelligence tools, such as Grammarly and ChatGPT, were used solely to enhance writing clarity and organization, not to generate or alter research data. Researchers

openly acknowledged any limitations or challenges encountered during the study to maintain integrity and transparency. Researchers shared the results with the academic community, relevant institutions, and participants to promote honesty, accountability, and trust in the study's findings on bystanders' apathy during emergencies.

J. Use of Patented or Copyrighted Materials

This study did not use any patented or copyrighted materials without proper authorization. If such materials were needed—such as published questionnaires, images, or other research tools- the researchers obtained formal permission and gave full credit to the original authors or creators. This ensured respect for intellectual property rights and maintained the academic integrity of the study on bystanders' apathy during emergencies.

K. Ethical Considerations for Human Participants

Since the study did not include animal testing, all ethical considerations focused on human respondents; each participant was treated with respect and dignity. Participation was voluntary, and respondents had the right to discontinue their involvement at any time without repercussions.

L. Responsible Use of AI and Other Related Technologies

In research, tools such as Grammarly, ChatGPT, Mendeley Reference Manager, and QuillBot have played a vital role in enhancing the research process. Grammarly improved the clarity and quality of our writing by pinpointing grammar and style mistakes. ChatGPT assisted us in generating ideas, summarizing concepts, and organizing both the paper and the questionnaires. Mendeley Reference facilitates source management, automatically creating citations and

ensuring accurate referencing. QuillBot helped paraphrase, making complex sentences more straightforward to understand and more concise. While these tools helped streamline our work, we used them responsibly to uphold academic integrity. We ensured they enhanced our research without replacing original thought or undermining the rigor of our study. By following ethical guidelines, we ensured that our work remained both efficient and academically robust.

M. Ethical Clearance and Institutional Approval

Researchers secure Approval from the appropriate institutional review board or ethics committee at Saint Michael College of Caraga. This Approval guaranteed that the research complied with all ethical standards and regulations established by the institution. Approval was requested and recorded before the start of the study, confirming that all aspects of the research met institutional ethical criteria.

2.7 Data Gathering Procedure

Prior to data collection, the researchers will prepare a formal letter addressed to the relevant authorities to seek permission to identify and access the target population, as well as to survey within the selected areas of Buenavista, Nasipit, and Carmen. Upon Approval, the researchers will personally distribute the survey questionnaires to the identified respondents.

Before answering the questionnaire, respondents will be informed about the purpose of the study, their voluntary participation, and the assurance of confidentiality and anonymity of their responses. Researchers will obtain Informed consent to ensure that participants fully understand their rights and the nature of their involvement in the study.

Participants will be given sufficient time to read and answer the questionnaire honestly. Once completed, the researchers will collect the completed questionnaires for checking and validation. The data will then be organized, encoded, and tabulated for statistical treatment using appropriate quantitative methods to determine the relationship between variables.

Finally, the analyzed results will be summarized, interpreted, and presented to the research panel and other concerned stakeholders for evaluation and dissemination.

2.8 Research Data and Analysis Plan

After collecting the survey questionnaires, the data will be carefully checked, coded, and tabulated to ensure accuracy and completeness. The gathered Information will be analyzed quantitatively using appropriate statistical tools to address each research question and test the study's hypotheses.

Descriptive statistics, including frequency, percentage, mean, and standard deviation, will be used to describe the demographic and experiential profiles of the respondents, as well as to summarize their responses to each item on the determinants of bystanders' apathy.

To determine the relationship between the independent variables (age, gender, educational attainment, prior first aid training, exposure to emergencies, and frequency of social media usage) and the dependent variables (diffusion of responsibility, evaluation apprehension, and pluralistic Ignorance), inferential statistical tests such as the Pearson Product-Moment Correlation and Regression Analysis will be applied.

The level of significance will be set at 0.05, and all statistical analyses will be conducted using a recognized statistical tool. The results will be presented in tables and interpreted in relation to the study's objectives and theoretical framework.

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